• DEMENTIA DIAGNOSIS

4 YEARS00 (S) 08/03 SA/H
Ref ID#: 4-0-00 Page:
Report: RC-TEST-DEMENTIA#RC-TEST-100

Dr. Robert Smith, MD
Neurology
1234 Medical Center Drive
Suite 567, Boston, MA 02115

PATIENT

Patient ID: RC-TEST-100
Name: SMITH,JOHN
DOB: 03/15/1952
Age: 73 years
Sex: Male

CHIEF COMPLAINT

Memory loss and confusion

HISTORY OF PRESENT ILLNESS

Mr. Smith is a 73-year-old right-handed male with a 3-year history of progressive memory impairment. He was referred by his primary care physician for evaluation of cognitive decline.

MEDICATIONS

Medication	Dose	Frequency
Donepezil	10 mg	Once daily
Memantine	20 mg	Once daily
Lisinopril	10 mg	Once daily
Atorvastatin	40 mg	Once daily
Aspirin	81 mg	Once daily

ALLERGIES

No known drug allergies

PAST MEDICAL HISTORY

Hypertension, hyperlipidemia, mild depression, prostate cancer (treated with radiation therapy in 2018).

FAMILY HISTORY

Mother: Alzheimer's disease, deceased at age 81.
Father: Myocardial infarction, deceased at age 68.
Brother: Alzheimer's disease, age 76.

SOCIAL HISTORY

Retired engineer. Married, lives with wife. Former smoker, quit 20 years ago. Occasional alcohol use.

REVIEW OF SYSTEMS

Constitutional: No fever, chills, or weight loss.
Neurological: Memory loss, confusion, occasional disorientation. No headaches, seizures, or focal weakness.
Psychiatric: Mild depression, no suicidal ideation.

PHYSICAL EXAMINATION

Vital signs: BP 138/82, HR 72, RR 16, Temp 98.4°F, SpO2 97% on room air.
General: Well-appearing male in no acute distress.
Neurological: Alert and oriented to person and place, oriented to month but not year. Mini-Mental State Examination (MMSE) score: 24/30. Deficits in delayed recall and serial 7s. Cranial nerves II-XII intact. Motor strength 5/5 bilaterally. Sensation intact to light touch. Deep tendon reflexes 2+ and symmetric. Gait steady, no tremor.

ASSESSMENT

Mild cognitive impairment, likely due to Alzheimer's disease, with early dementia.

PLAN

1. Continue donepezil 10 mg daily and memantine 20 mg daily.
2. Order MRI brain without contrast to rule out structural lesions.
3. Repeat neuropsychological testing in 6 months.
4. Discuss driving safety with patient and family.
5. Follow up in 3 months.

Electronically signed by Dr. Robert Smith, MD
Date: 03/15/2025 10:42 AM